Sleep regimen — old stack vs. new

A medium-term choice, honestly framed, with clear stacking rules · 2026-05-21

For Dad, Mom, Leah, David, Debbi, family team

What this is for. The actual decision Dad and Mom are weighing on sleep — not just for tonight but for the medium-term window before ECT starts at U-M. The two real candidates are **lemborexant 5 mg + melatonin 10 mg** (the new agent Julane recommends) and **doxepin 6 mg + trazodone 50 mg + melatonin 10 mg** (the prior stack Dad was on, which produced 5.5–6 hours of "okay not great" sleep). Each is defensible. The choice depends on how the family weighs known-tolerated-but-unspectacular against untried-but-with-real-evidence-and-a-better-medium-term-profile. This handout lays out both honestly, plus the clear "do not combine" stacking rules so the choice is made deliberately rather than by drift.

BACKGROUND THAT HOLDS ACROSS EITHER CHOICE

- **Cymbalta 60 mg daily** — taken in the morning. Taper is paused; Dad is back at 60 mg as of 5/21 after the difficult overnight 5/20.
- **Depakote status TBD** — being discussed; not yet confirmed as part of the medium-term regimen. This handout treats Depakote as a separate decision and does not fold it into the sleep regimen question.
- **Melatonin 10 mg HS** — continues with either sleep regimen. Negligible risk; modest evidence; no interactions.
- **Lorazepam (Ativan) PRN** — available per Maxner clearance. Family preference is to avoid scheduled use given habit-formation concerns; a single PRN dose for an acute episode is acceptable.
- **ECT at U-M** is 2–4 weeks out. The medium-term window for the sleep regimen is approximately the next 2–4 weeks until ECT starts and then through the ECT course.

THE TWO REAL OPTIONS

Both are defensible. The honest read is that this is closer to a coin flip than a clear winner. Read the pros and cons together and pick the one that better fits Dad's preferences, the medium-term trajectory, and the U-M Movement Disorders evaluation that's pending.

OPTION A — THE NEW STACK (JULANE'S RECOMMENDATION)

Lemborexant 5 mg HS + melatonin 10 mg HS

PROS

- **Real evidence base in elderly insomnia.** SUNRISE-1 (Rosenberg 2019, JAMA Network Open) showed improved sleep onset and sleep maintenance in adults ≥55 vs. placebo and zolpidem. SUNRISE-2 confirmed durable effect over 12 months.
- **17-hour half-life provides built-in sleep maintenance.** The drug is still working at 2 AM — reduces wake-after-sleep-onset (WASO) in the second half of the night, which is precisely where Dad's prior stack underperformed (waking unrested).

OPTION B — THE OLD STACK

Doxepin 6 mg HS + trazodone 50 mg HS + melatonin 10 mg HS

PROS

- **Known-tolerated by Dad.** Personal data: 5.5–6 hours of sleep, no serious adverse effects, no overnight emergencies. The known floor is meaningful in a fragile moment.
- **Both components have AGS-Beers-endorsed elderly indications.** Low-dose doxepin (3–6 mg) is one of the few hypnotics specifically recommended in the 2023 Beers companion as a safer alternative; trazodone 50 mg is widely used at this dose even though formal evidence is weaker.

- **SELENADE 2025 evidence in comorbid depression:** improved both insomnia and depressive symptoms over 12 weeks, with HAM-D-17 decreasing by 5–6 points. Directly applicable to Dad's profile.
- **No anticholinergic burden** — important given Dad's prodromal Lewy-spectrum markers (formal RBD 8.5 years, gait abnormality, constipation). Doxepin's burden is small at 6 mg but isn't zero, and cumulative exposure matters over months.
- **No dependence risk, no withdrawal pattern.** Lowest abuse-potential profile of any insomnia agent per FAERS data — lower than trazodone and doxepin.
- **Single-agent simplicity.** One sleep agent + melatonin is easier to track, attribute, and adjust than three concurrent agents.

CONS

- **Untried by Dad.** No personal data on whether it works for him. First-night and first-week efficacy in trials doesn't guarantee his individual response.
- **Cost.** ~$300/month not covered by insurance. Family has dismissed this as a barrier; flagging for completeness.
- **Not anxiolytic per se.** Lemborexant blocks the wake signal but does not directly reduce anxiety. If Dad goes to bed anxious, the orexin blockade alone may not be enough to override it.
- **Theoretical RBD consideration.** Lemborexant increases REM sleep, which could in theory increase dream-enactment frequency. No clinical evidence this has materialized in any patient cohort; bed safety (no nearby hard edges) is a reasonable precaution.
- **Nocebo risk under acute anxiety.** Starting a new agent at a fragile moment carries some risk that a poor first night gets catastrophized into "doesn't work" before the agent has been fairly tested.

- **Doxepin 6 mg has FDA approval specifically for insomnia.** At this dose it's primarily a selective H1 antihistamine; the anticholinergic, serotonergic, and noradrenergic effects that drive antidepressant-dose doxepin's side-effect profile are minimal at 6 mg.
- **Trazodone adds sleep-onset coverage** via 5-HT2A/alpha-1/H1 antagonism; doxepin adds sleep-maintenance coverage. The combination addresses both halves of the night.
- **No new variable.** Choosing this preserves the family's ability to attribute any changes in Dad's state to Cymbalta, Depakote (if it ends up on board), or other factors — without lemborexant as an additional confound.
- **Lower nocebo risk.** Dad knows this regimen; the anxiety around "will the new thing work" doesn't apply.

CONS

- **"Okay not great" is the realistic ceiling.** 5.5–6 hours, no dream recall, wakes unrested, can't nap during the day. The known floor isn't a robust one; this regimen has not produced the sleep Dad actually needs.
- **Anticholinergic burden over time.** Doxepin at 6 mg is at the low end of the dose-response curve, but it isn't zero. Over months, cumulative anticholinergic exposure is the wrong direction in a patient with formal RBD + gait abnormality + constipation (prodromal Lewy-spectrum markers). Acute risk for one night or one week is negligible; the medium-term direction matters.
- **Dad attributed "agitation" to doxepin.** The published literature has no documented paradoxical-agitation pattern at 6 mg, so this is most likely attribution error or coincidence — but Dad's subjective experience is itself a clinical fact that affects adherence.
- **Doesn't advance the long-term regimen.** U-M will eventually want a sleep stack going into ECT and beyond; doxepin will likely come off at some point given the prodromal markers. Choosing this now defers that transition.
- **Three concurrent sleep agents (counting melatonin) is more attribution-noisy** than one. If anything changes in Dad's state — better or worse — it's harder to know which agent is responsible.

How they compare at a glance

Dimension	Option A — Lemborexant + melatonin	Option B — Doxepin + trazodone + melatonin
Personal data for Dad	None yet — untried	5.5–6 hrs sleep, no dream recall, wakes unrested, no serious AEs
Trial evidence in elderly	Robust — SUNRISE-1, SUNRISE-2, SELENADE 2025 (depression comorbidity)	Doxepin 6 mg: robust (FDA-approved for insomnia, multiple RCTs). Trazodone 50 mg: weak (AGS Beers companion: "insufficient evidence")
Sleep maintenance (2 AM problem)	17-hr half-life — drug still active overnight	Doxepin's primary benefit is sleep maintenance; combination covers both halves
Anticholinergic burden	None	Modest (doxepin); concerning over months in prodromal Lewy-spectrum
Dependence risk	None — FAERS lowest of any insomnia agent	None — both have very low abuse potential
Anxiolytic effect	None directly — blocks wake signal but not anxiety	Modest — doxepin's antihistamine effect calms; trazodone's 5-HT2A/alpha-1 antagonism is mildly anxiolytic
Cost (Dad-side)	~$300/month not covered	Both generic; trivial cost
Number of agents	1 (+ melatonin)	2 (+ melatonin)
Medium-term direction	The agent Julane recommends and U-M will likely continue through ECT	Anticholinergic-loaded; U-M will likely want to transition off doxepin at some point
First-night risk	Moderate — untried; nocebo risk if framed poorly	Low — known-tolerated; predictable

Clear stacking rules — what NOT to combine

DO NOT — THESE COMBINATIONS ARE NOT THE PLAN AND SHOULD NOT BE DRIFTED INTO

- **DO NOT take lemborexant AND trazodone the same night.** They are alternatives, not stack-mates. If choosing Option A, both doxepin and trazodone are off the regimen. If choosing Option B, lemborexant is off. Stacking two sedating agents with different mechanisms adds risk (orthostatic hypotension, falls, additive sedation) without an evidence base for the combo.
- **DO NOT take lemborexant AND doxepin the same night.** Same logic — Option A is lemborexant only (+ melatonin). Adding doxepin reintroduces anticholinergic load and undermines the rationale for switching.
- **DO NOT take only one of doxepin or trazodone (Option B is the pair).** Trazodone monotherapy has weak evidence in elderly (AGS Beers: "insufficient evidence"). Doxepin monotherapy at 6 mg works on sleep maintenance but is light on sleep onset. The Option B value is the combination; don't degrade it by dropping one component.
- **DO NOT fold Depakote into the sleep regimen question.** Depakote is a separate decision being discussed for acute anxiety. Whether it goes on board, at what dose, and for how long is a question for the U-M conversation. Treating Depakote as part of the sleep stack confuses two independent questions.
- **DO NOT take Ativan as a scheduled nightly dose.** One emergency use is fine; nightly use establishes a dependence pattern that becomes hard to undo. If Ativan ends up being needed multiple nights in a row, that's a signal to escalate to U-M faster, not to add it to the routine.
- **DO NOT stack a second sleep aid mid-night.** If the chosen regimen doesn't work and Dad wakes at 12 or 1 AM, do not pile on a different sleep aid. Stimulus control + acceptance is the right response, not pharmacologic escalation in the middle of the night.
- **DO NOT skip the Cymbalta morning dose to "reduce serotonergic load."** The taper is paused. Dad is at 60 mg by design. Skipping a dose restarts the destabilization the pause is meant to prevent.

DO — THESE COMBINATIONS ARE THE PLAN AND ARE EVIDENCE-SUPPORTED

- **DO take Cymbalta 60 mg AM** per the paused-taper schedule. Sleep regimen is separate.
- **DO take melatonin 10 mg HS** with either Option A or Option B. Negligible interactions; modest sleep-onset support.
- **DO commit to one regimen for a fair trial window.** Option A needs 3–4 nights minimum to evaluate; Option B already has Dad's prior data. Switching mid-week defeats the assessment.
- **DO use Ativan PRN as a single-dose rescue if truly needed** — once. Note the dose, time, and effect. Don't escalate to scheduled use without escalating to U-M.
- **DO track sleep nightly in writing** — onset, awakenings, total time, subjective quality, AM mood. The data is what informs the next decision.
- **DO treat Depakote, the Cymbalta taper, and the sleep regimen as three independent questions** — each gets discussed with U-M on its own merits.

Tonight specifically

Tonight is the first night of whichever option is chosen. If Option A, it's the first lemborexant trial. If Option B, Dad goes back to a known stack. Either way, the framing for Dad pre-bed matters more than usual, given the acute anxiety state today.

> **FOR MOM TO DAD — IF OPTION A IS CHOSEN**
>
> *This is the medication Julane recommended. It works differently from doxepin and trazodone — instead of making you sleepy, it turns down the wake signal. It lasts the whole night, so if you wake up at 2 AM, the medicine is still working — give it a few minutes. We're trying it for the first time tonight, so it may take a few nights to know how well it works for you. One night doesn't tell us much; we'll know more after three or four. If you really can't sleep, we have Ativan as a one-time backup.*

> **FOR MOM TO DAD — IF OPTION B IS CHOSEN**
>
> *We're going back to the regimen you were on before — doxepin, trazodone, melatonin. You know how this feels and we know it gives you a decent night's sleep. We're not committing to it long-term — when U-M gets involved we'll revisit. Tonight is about familiarity and not adding a new variable on a hard day.*

How we'll know what to do over the next 1–2 weeks

- **Whichever option is chosen, give it a fair trial window** — 3–4 nights minimum for Option A; Dad already has data on Option B so 1–2 nights is enough to confirm it's still consistent with his prior experience.
- **What "working" looks like:** ≥6 hours of total sleep, falls asleep within ~30 min, no more than 1–2 awakenings of ≤20 min each, wakes feeling at least somewhat rested, no new daytime symptoms (worse anxiety, motor changes, cognitive cloudiness beyond baseline).
- **What "not working" looks like:** ≤5 hours total, multiple long awakenings, AM anxiety spike, new daytime symptoms.
- **If Option A is chosen and works:** stay on it. Bring it to the Maxner conversation as the medium-term sleep stack going into ECT.
- **If Option A is chosen and doesn't work after 3–4 nights:** revert to Option B; flag for Maxner that lemborexant didn't deliver. Don't abandon lemborexant after one bad night.
- **If Option B is chosen:** the question revisits with U-M during the Maxner conversation. The case for transitioning to lemborexant pre-ECT is on the table; the family hasn't ruled it out.
- **If anything new appears** — dream enactment / RBD escalation, new motor symptoms, paradoxical agitation, GI changes — note it. These observations feed the Movement Disorders evaluation.
- **If Ativan is used:** note the dose, time, and how much it helped. One night is fine; multiple nights in a row is a signal to escalate to U-M, not to extend the pattern.

Bottom line

This is closer to a coin flip than a clear winner. **Option A** has the better medium-term direction — real evidence in elderly, no anticholinergic burden, single-agent simplicity, the agent U-M and Julane will likely want. **Option B** has the better short-term predictability — known-tolerated, no nocebo risk, modest anxiolytic effect from the antihistamine + 5-HT2A antagonism. Dad and Mom are the right people to make this call. The job of this handout is to ensure the call is deliberate, that whichever option is chosen gets a fair trial, and that the family doesn't drift into a regimen no one chose — by stacking lemborexant on top of doxepin + trazodone, by adding Depakote without a separate conversation about it, or by escalating Ativan into nightly use without anyone noticing.

The medium-term frame matters because ECT is 2–4 weeks out. Whichever stack is on board going into Maxner's intake conversation becomes the starting point for U-M's plan. That's worth choosing on purpose rather than by accumulation.